

DETAILED INVESTMENT PROPOSAL

Concept Note

The full case for the EECA Lung Health Hub – a phased, evidence-based investment across nine countries in Eastern Europe & Central Asia.

DETAILED INVESTMENT PROPOSAL: THE EECA LUNG HEALTH HUB

From Decision to Delivery: Advancing Political Certainty and Lung Health Security in Eastern Europe and Central Asia

Last updated: September 2026

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Revision for review — September 2026. Everything marked in **red** changed in this round. Black text is unchanged. Please comment directly on the red passages.

This round did three things: it acted on the team's nine comments, it corrected every figure that could not be supported by a published source, and it added a references section — the note previously cited nothing at all.

Removed, and so not visible below. The claim of "approximately 300,000 vulnerable patients ... a conservative estimate based on the annual incidence of TB alone in the target countries" is gone: WHO/ECDC record about 225,000 estimated TB cases in the *entire* WHO European Region in 2023, so the figure was roughly three times too high. The figures "85% of the TB burden" and "99% of MDR-TB" are gone: they describe the WHO European Region rather than EECA, and the 99% appears nowhere in the current WHO/ECDC report — it survives only in a 2007–2015 plan. Two references were dropped because nothing in the note cited them (Impakter, on US philanthropy; Gavi 6.0, on vaccines — whose link was also dead). Two claims were softened because no source exists for them: the statistic about an "average MP" facing a deluge of legislation, and, on the website, "22 of 26 high-burden countries that raised domestic TB budgets had an active caucus".

IN BRIEF: TURNING POLITICAL DECISIONS INTO DELIVERED CARE, FASTER

The **Eastern Europe Central Asia (EECA) Lung Health Hub** is an action-oriented regional platform that helps countries turn political decisions on lung health into real action – faster and more effectively. It focuses on lung health as an entry point for strengthening health systems across Eastern Europe and Central Asia, combining policy intelligence, financing and implementation support.

Rather than creating another standalone initiative, the Hub is designed to work through existing national institutions and regional partnerships, helping countries turn evidence and commitments into funded, operational solutions.

EECA Lung Health Hub works through three connected functions:



Engine

An AI-supported tool that analyses legislation, helps prepare amendments, and speeds up parliamentary work on lung health – with dedicated human oversight ensuring coordination.



Bridge

A platform for direct regional parliamentary cooperation. It identifies common priorities and successful approaches across countries, and makes regional exchange more practical and accessible.



Shield

An early-warning and continuity system that alerts decision-makers to risks that could interrupt treatment, and supports practical measures to reduce differences in access to medicines.

The Hub starts with a *six-month Foundation Phase* in one country to test and prove the model, before expanding regionally. It works through existing institutions rather than creating parallel structures.

EXECUTIVE SUMMARY

[the tagline "A Catalytic Two-Year Programme" was removed]

A multi-sectoral programme that includes the development of an artificial intelligence application to help the region move more quickly from decisions to real results. The programme will provide more open access to the decision-making process for parliamentarians, civil society and affected communities. It also includes a system that uses regional data to identify where patients may lose access to treatment – and alerts decision-makers early.

The Eastern Europe and Central Asia (EECA) region faces a critical juncture. Persistent health epidemics, escalating geopolitical complexities, and a fundamental shift in global health financing demand a new paradigm: rapid, decisive action and national ownership. With major donors scaling back their funding commitments globally – and particularly across EECA since early 2025 – the region must urgently adapt to safeguard its health future.

This proposal focuses on lung health diseases – the leading cause of preventable deaths and a key driver of health system strain across the region, where tuberculosis, chronic respiratory conditions, and post-infection complications continue to threaten population health and economic stability.

This Detailed Investment Proposal outlines a strategic opportunity to support the EECA Lung Health Hub as a full-scale, high-impact program, **built as a phased programme over two years, opening with a \$100,000, six-month Foundation Phase in one EECA country that delivers a fully working solution [was "structured ... over 2 years: a USD 100,000, 6-month Foundation Phase". "Built" per comment; "\$" replaces "USD" throughout; this is now the only statement of duration in the executive summary]**, followed by country-by-country replication and regional build-out, **to a total programme value of \$1.2 million**. Full phased budget and terms are available on request.

This Hub is designed as a fast, flexible engine to move from decision to delivery, giving politicians greater certainty and speeding up the rollout of essential lung health policies and services. It aims to tackle the common problem of "too many laws vs. too little implementation", which often slows progress in the region.

Crucially, the Hub is developed with support from the **Global TB Caucus**, a global network of parliamentarians working to drive political action, improve policies, and secure funding to end tuberculosis. *Its regional network for Eastern Europe and Central Asia has been active since 2016.³ [was "active in the EECA region since 2014". The Global TB Caucus was founded globally in 2014; its Eurasian Parliamentary Group on TB was established in June 2016]* This existing network, comprising nine active national caucuses (Armenia, Azerbaijan, Georgia, Kazakhstan, Kyrgyzstan, Moldova, Tajikistan, Ukraine, Uzbekistan), offers exceptional regional reach for rapid, sustainable impact. Working with this political reach and trust, the Hub will leverage advanced digital and AI-enabled solutions to accelerate the translation of policy decisions into implementation. This will include tools such as AI-assisted legislative drafting and analysis (with mandatory human review) for MPs and a "Last-Mile Logistics Dashboard" for efficient distribution. Together, these innovations aim to significantly shorten the time between decision-making and execution, with the potential to reduce timelines to as little as one week in optimized settings.

Functioning as a **Health Security Coordination Center**, the Hub is designed to protect continuity of care where the regional burden is heaviest. The nine partner countries include three of the six highest-burden countries in the WHO European Region: Ukraine (42,000 estimated cases in 2023), Uzbekistan (20,000) and Kazakhstan (14,000).¹ *(Replaces the 300,000 claim. These are published per-country figures, quoted rather than summed.)*

This proposal details a high-impact pathway to strengthen lung health sovereignty, regional resilience, and sustainable national health systems across EECA, offering national partners, donors and private stakeholders a strategic, measurable, and timely investment in the region's health and stability.

1. THE OPPORTUNITY: NAVIGATING A NEW REALITY WITH URGENCY AND PRECISION

The EECA region is currently facing overlapping health, economic, and geopolitical challenges that demand a strategic and locally-led response.

1.1. Persistent Lung Health Crises: A Focused Imperative

The WHO European Region recorded an estimated 225,000 new and relapse tuberculosis cases in 2023, about 84% of them in the 18 countries WHO designates as high-priority – a group that includes all nine EECA countries this programme works with. Incidence across those 18 averages 46 per 100,000, five times the EU/EEA average. The Region also carries the world's heaviest drug-resistant burden: an estimated 65,000 rifampicin-resistant and multidrug-resistant (RR/MDR-TB) cases, and nine of the 30 countries with the highest MDR-TB burden worldwide.¹ [every figure here is quoted from the WHO/ECDC report and now carries a reference] Beyond TB, other critical lung health conditions, such as chronic respiratory diseases and emerging respiratory infections, further strain already fragile health systems. The Hub will focus on lung health diseases, reflecting both their disproportionate burden among key and vulnerable populations, and the high potential for impactful, targeted responses.

225,000

TB cases, WHO European Region (2023)

84%

in the 18 high-priority countries

65,000

drug-resistant (RR/MDR-TB) cases

1.2. The Global Fund Transition: A Call for National Ownership

Major donors are substantially reducing their active engagement in the EECA region from early 2025 onward.² [now sourced] This strategic pivot creates a critical funding gap and underscores the urgent need for EECA countries to transition towards sustainable national-level domestic financing for health. International donors are increasingly advocating for national ownership and the development of

robust local funding mechanisms, placing the onus on regional advocates to secure and optimize internal resources. This transition presents a unique opportunity for national donors to step forward and champion a model of self-reliance. This proposal offers a concrete, ready-to-implement pathway to operationalize that shift through practical mechanisms and milestones, as well as demonstrable regional impact.

1.3. Geopolitical Instability and Health Security: The Unseen Crisis

Escalating geopolitical instability and security threats across the region, including continued full-scale aggression from Russia in Ukraine and other geopolitical threats beyond the region, compound the crisis. The decline in global health donor spending further exacerbates this vulnerability, particularly as the shrinking civic space and new restrictive legislation increasingly threaten people's safety. In the event of escalating conflicts or other geopolitical shocks, the Hub's proposed role as a Health Security Coordination Center becomes paramount, ensuring uninterrupted, coordinated assistance to all affected patients, regardless of the security context. This is a direct investment in regional stability and health security.

1.4. Decision Paralysis and Implementation Lag: The Regional Hesitation

Despite increasing health expenditures in many EECA countries, budgets often do not match the real disease burden because planning is fragmented, spending data are weak, and forecasting is limited. A significant challenge is often observed as "regional hesitation", where political systems struggle to move from commitment to concrete action on complex health issues. **It also stems from the legislative workload carried by parliamentarians: where health policy competes with a heavy flow of other legislation, the capacity for swift, informed decision-making on complex health questions is constrained. [softened — the previous wording implied a statistic about "an average MP" that no source supports]** This leads to decision paralysis and implementation lag, where critical health policies take months, if not years, to translate into tangible benefits for the population. Without robust data governance, standards, and privacy safeguards (aggregated / non-identifiable data only; ISO 27001-grade security; GDPR-equivalent protection; sovereign / in-region hosting; independent audit), the potential of digital health remains untapped, risking donor skepticism about "AI breakthroughs."

2. THE SOLUTION: THE EECA LUNG HEALTH HUB

The EECA Lung Health Hub is designed as a lean, high-impact Full-Scale Program, strategically focused on catalyzing political will, community empowerment, and data-driven policy reform within a \$1.2 million budget. ["over 2 years" removed as a duplicate statement of duration] Our approach is built on the principle of regional ownership and sustainability, leveraging existing strengths and addressing critical gaps.

2.1. Support from the Global TB Caucus Network

Unlike new initiatives that require extensive foundational work, the Hub works with support from the Global TB Caucus. Its regional network for Eastern Europe and Central Asia has been active since 2016, cultivating strong relationships and a track record of mobilizing political commitment for lung health.³ The Hub collaborates with nine active national TB caucuses in Armenia, Azerbaijan, Georgia, Kazakhstan, Kyrgyzstan, Moldova, Tajikistan, Ukraine, and Uzbekistan. These established relationships provide an exceptional platform for collaboration, rapid engagement and impact, significantly reducing start-up time and costs.

2.2. Lean, Autonomous Regional Governance Structure

The Hub will operate as an autonomous, regionally-led center, ensuring that decisions are made by and for the EECA context, with support from the Global TB Caucus network. This lean structure is designed for agility, direct impact, and fostering regional ownership and accountability. It avoids bureaucratic overhead, allowing for efficient allocation of the \$1.2 million budget directly to programmatic activities.

Governance Model:

Governance rests on four operating bodies, coordinated by a **Regional Steering Committee** of Parliamentary Council, Civil Society Council, and regional technical-expert representatives, which sets strategic direction and ensures alignment with regional priorities:

Parliamentary Council

Formed with representatives from the nine national TB caucuses (Armenia, Azerbaijan, Georgia, Kazakhstan, Kyrgyzstan, Moldova, Tajikistan, Ukraine, Uzbekistan), this council will champion legislative and policy reforms, advocate for domestic resource mobilization, and ensure political buy-in at the highest levels. Their established relationships and influence are critical for translating policy into action.

Advisory Board of Former MPs (Ex-MPs)

A specialized body of former parliamentarians with deep experience in health policy and legislative processes. This board will provide strategic mentorship to current caucus members, lead high-level diplomatic missions, and offer a "layer of certainty" through peer-to-peer political guidance.

Civil Society Council

Comprising representatives from key populations and civil society organizations across the EECA region, this council will ensure that the Hub's activities are community-led, rights-based, and responsive to the needs of vulnerable populations. It will provide ethical oversight, particularly for data use, and ensure accountability and transparency. The Council will work at the regional, national, and community levels, with key roles led by community-led representative networks and input from civil society gathered through regular community consultations.

Technical Secretariat

A small, agile team responsible for day-to-day operations, technical support, and efficient implementation of the program pillars. The Secretariat will include three core positions: Lead for Parliamentary Engagement, Lead for Civil Society Engagement, and AI & Systems Architecture Lead. In addition, several support operations positions will be included, such as finance, administration, communications, and AI & Systems Architecture Assistant.

This structure ensures that the Hub is deeply embedded within the regional political and social fabric, enabling swift, context-specific decision-making and implementation, with support from the Global TB Caucus and its regional network.

2.3. The \$1.2 Million Programme: The EECA Lung Health Hub ["(2-Year Program)" removed]

Component 1: The Engine (AI-Powered Legislative Platform)

Key function: Turn political decisions into ready-to-table legislation and budget action, faster implementation.

KEY DELIVERABLES ["(WITHIN 24 MONTHS)" REMOVED HERE AND ON THE OTHER TWO COMPONENTS]

- Secure MP Portal – role-based, invitation-only workspace to draft, compare and share laws & amendments
- AI-assisted drafting & amendment analysis – generates options and flags inconsistencies, with mandatory human review (AI is an enabling tool, not the decision-maker)
- Comparative-Law Intelligence – searchable, multi-country EECA health-legislation database
- Automated stakeholder & timeline mapping – MPs, committees, legislative windows
- Integrated community feedback loop
- No personally identifiable data; full audit trail

Result / value: Reduces routine legislative turnaround from months toward days – with traceable, governed, human-reviewed outputs.

Component 2: The Bridge – AI-Driven Diplomacy & Political Coordination

Key function: Promote targeted, evidence-driven political engagement and regional cooperation instead of ad hoc efforts.

KEY DELIVERABLES ["(WITHIN 24 MONTHS)" REMOVED HERE AND ON THE OTHER TWO COMPONENTS]

- Country-matching analytics – recommends bilateral cooperation from budgets, legislation, outcomes & readiness (AI-assisted, human-validated)
- Evidence-based bilateral & multilateral missions
- Regional Parliamentary Summits
- Decision Briefs (2–4 pages) for policymakers
- Budget & Policy Intelligence – budget reviews, costing models, resource mapping, spending alignment

Result / value: Turns diplomacy into a precision tool that links engagement to concrete policy and financing outcomes.

Component 3: The Shield – Health Security, Crisis Response & Continuity of Care

Key function: Keep care continuous and decision-makers informed during potential crises.

KEY DELIVERABLES ["(WITHIN 24 MONTHS)" REMOVED HERE AND ON THE OTHER TWO COMPONENTS]

- Regional health-security coordination protocol
- Continuity-of-care mapping (aggregated, non-identifiable data)
- Signal-based early-warning alerts for MPs (supply, displacement, funding disruptions)
- Interactive dashboards – disease burden + financing, regional/country views
- Supply & last-mile visibility dashboard – a data/visibility layer over supply status (not physical logistics execution)
- Privacy-by-design; aggregated indicators only

Result / value: Protects continuity of care and gives early, governed warning before care is interrupted.

Together these present the Hub as a single, integrated system rather than three separate projects. The Coordination Hub provides the backbone, the Engine accelerates policy action, the Bridge aligns diplomacy and financing, and the Shield protects health system continuity under stress. Together, these components are designed to move the region from political commitment to practical implementation much faster than conventional approaches.

All three components run on a shared, governed data layer – a common indicator model, interoperability standards (aligned with the WHO global strategy on digital health⁴), validation/versioning, ISO 27001-grade security and sovereign / in-region hosting – which is what lets the Engine, Bridge and Shield share data safely.

3. THE INVESTMENT THESIS: HIGH-LEVERAGE, RAPID IMPACT FOR NATIONAL DONORS & PRIVATE PARTNERS

This \$1.2 million programme establishes a practical, evidence-based decision-support system for lung health in EECA – designed to help governments act on evidence, spend more efficiently, and maintain system continuity during a critical transition period.

With the regional withdrawals in 2026, the Hub serves as a bridge to national health sovereignty, ensuring that political commitments translate into sustained domestic action.

3.1. The Engine – Policy Certainty and Immediate Action

This component directly addresses one of the biggest structural gaps in the region: the absence of clear, actionable intelligence on how health resources are allocated and how policy decisions translate into financial commitments.

By equipping ministries, parliaments, and national stakeholders with Budget Review Reports, Resource Mapping Analyses, Policy Costing Models, and AI-assisted legislative drafting, the Engine enables:

- evidence-based decisions on domestic health financing
- credible costing of policy and legislative options
- stronger alignment between policies and national budgets
- reduced dependency on external technical assistance

Why it's investable: It strengthens core state functions – lawmaking, budgeting, and financial analysis – linking policy intent directly to realistic fiscal commitments and helping governments reduce inefficiencies while prioritizing high-impact interventions.

Impact: Political decisions move from concept to executable, financially grounded policy within days, providing unprecedented certainty and speed.

3.2. The Bridge – Smart Diplomacy and Financing Alignment

This layer ensures that regional collaboration and political engagement are no longer ad hoc, but strategically optimized through data, evidence, and structured accountability.

Working with support from the Global TB Caucus network and by linking budget data, legislative progress, and health outcomes, the Bridge:

- strengthens parliamentary engagement, oversight, and structured dialogue between government, civil society, and technical actors
- ensures missions and political platforms result in concrete policy, financing, and implementation outcomes
- provides budget advocacy toolkits, oversight systems, accountability scorecards, and Legislative and Budget Impact Notes

Why it's investable: It transforms existing political platforms (e.g., parliamentary networks) from advocacy spaces into effective delivery and accountability instruments, increasing transparency and ensuring funding decisions translate into real outcomes.

Impact: Regional cooperation and parliamentary processes become precision tools for accelerating national reforms, resource optimization, and sustained implementation.

3.3. The Shield – Health Security and System Resilience

In a region facing geopolitical instability and migration pressures, this component establishes practical operational intelligence that transforms fragmented data into usable insight for planning, forecasting, and continuity of care.

By integrating interactive dashboards, burden-versus-spending analyses, AI-supported forecasting models, and coordination mechanisms, the Shield:

- protects continuity of care and prevents treatment interruptions and drug resistance
- forecasts resource needs (drugs, diagnostics, service demand) and identifies inefficiencies

- enables rapid, coordinated crisis response and earlier risk identification

Why it's investable: It directly contributes to national stability, public trust, and health security by strengthening the capacity to monitor system performance, anticipate operational risks, and make coordinated resource decisions – without reliance on complex infrastructure.

Impact: Health systems become adaptive, crisis-ready, and capable of protecting vulnerable populations even in unstable conditions.

The Bottom Line: Rapid Execution and Decision Support

This is not a traditional program. It is a regional execution system that:

- converts evidence into actionable financing and policy decisions
- connects budget intelligence, operational monitoring, parliamentary accountability, and implementation tracking into one coherent system
- demonstrates a replicable model for national health sovereignty in the EECA region

Return on investment:

- Faster, financially grounded policy implementation
- Stronger planning, resource optimization, and continuity of care during the funding transition
- More transparent, accountable, and crisis-resilient health systems
- **Demonstrated decision value in the lead partner country [was "across 2–3 pilot countries", which contradicted the one-country Phase 1 stated everywhere else], with a clear roadmap for regional scale**

Innovation: The Rapid Execution Cycle

The core innovation of the EECA Hub is the full integration of the Engine, Bridge, and Shield into a single, seamless execution flow – dramatically shortening the gap between political decision and tangible implementation.

- **Engine (Policy & Budget Intelligence):** An MP or ministry official drafts or amends legislation using AI-supported tools, with real-time access to budget analysis, policy costing, legal comparisons, and stakeholder input.

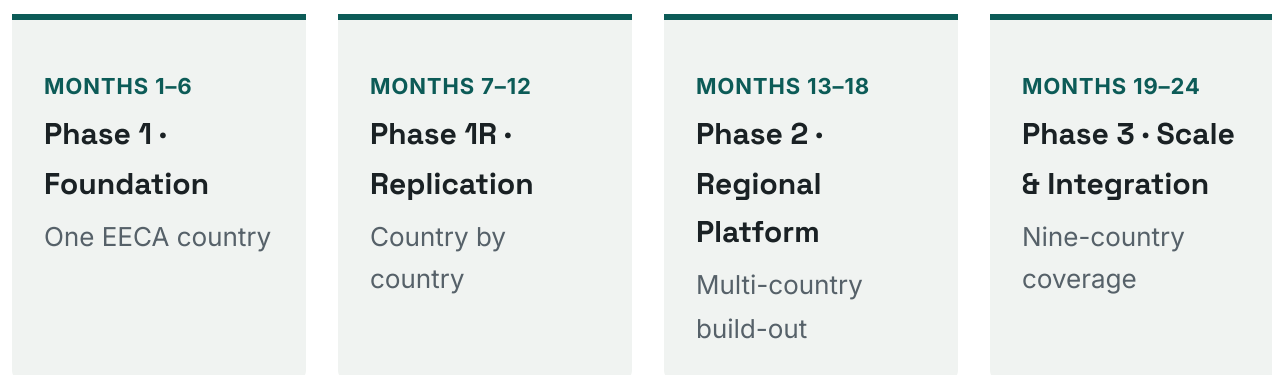
- **Bridge (Smart Diplomacy & Accountability):** The system automatically identifies relevant experiences from other countries, activates targeted parliamentary and political engagement, and generates supporting materials (budget impact notes, advocacy briefs, accountability scorecards).
- **Shield (Operational Intelligence & Health Security):** Approved decisions are immediately translated into operational dashboards, resource forecasting, procurement signals, and continuity plans – ensuring rapid operational follow-through.

Result: A fully connected decision-to-delivery loop that compresses what traditionally takes months (or even years) of fragmented efforts into a matter of weeks. Instead of disconnected initiatives, governments get an integrated system where evidence, political will, and operational capacity reinforce each other in near real-time.

4. MECHANICS OF PROJECT IMPLEMENTATION

Sections 4 and 5 were proposed in the previous review edition and carried no comments, so they are treated as accepted and are now part of the note. The month ranges appear only in the timeline graphic; the four phase headings below no longer repeat them.

The programme is implemented in phases. Each phase is independently fundable, delivers a working result on its own, and the foundation unit is designed to replicate country-by-country – a donor never funds a promise, only a repeat of something already proven.



Phase 1 – Foundation: One EECA Country

Objective: a working solution, live, in one country. The lead partner country is selected from the nine active national TB caucuses on criteria including caucus readiness, political momentum, and donor fit. Delivered at the end of the phase: lean governance established; a Legislative Ask Map (the exact regulatory changes needed in the lead country, validated with MPs and communities); the Hub App core on sovereign infrastructure (roles/authentication, country profile, stakeholder directory, caucus workspace); a screening-intelligence dashboard with a national-reporting interoperability pilot; and a caucus legislative workplan running in the App – the proof unit.

Phase 1R – Country Replication

The identical playbook stamped into additional caucus countries, each producing its own Legislative Ask Map, dashboard and caucus workplan. Replications run in

parallel.

Phase 2 – Regional Platform Build-Out

Full Sovereignty App: Secure MP Portal, Comparative-Law Intelligence (multi-country EECA database), AI-assisted drafting & amendment analysis with mandatory human review; EN/RU multilingual rollout; hardened security posture (ISO 27001-grade, sovereign hosting); first Regional Parliamentary Summit on the platform; Decision Briefs and budget/policy intelligence production.

Phase 3 – Scale & Integration

Onboard the remaining caucus countries (full nine-country coverage); full Bridge (country-matching analytics, evidence-based missions, accountability frameworks); full Shield (signal-based early warning, continuity-of-care mapping, supply & last-mile visibility dashboard; private-sector integration); comprehensive evaluation, sustainability & domestic-financing strategy, long-term institutionalization.

The components in §2.3 (Engine · Bridge · Shield) are the *what*; the phases are the *when and how much*: the Engine's core ships in Phase 1 and completes in Phase 2; the Shield's intelligence layer seeds in Phase 1 and completes in Phase 3; the Bridge operates from Phase 2 and completes in Phase 3.

5. BUDGET STRUCTURE

[shortened per comment: detail on request, then how the money splits]

The programme is funded phase by phase, with full line-item detail and terms available on request. The chart below shows how the budget is divided across the four programme pillars.

WHERE THE MONEY GOES

A phased, transparent allocation.

Indicative allocation across the four programme pillars. Full line-item budget shared with partners in confidence.



Indicative allocation. Full phased budget available on request.

Please [contact us](#) or start a [consultation](#) to receive the detailed budget package.

6. CONCLUSION: A STRATEGIC INVESTMENT IN NATIONAL HEALTH AND REGIONAL STABILITY

The EECA Lung Health Hub closes a critical gap: turning political decisions into real delivery of care – fast – entered through a Foundation Phase in one EECA country that delivers a working solution before any further funding is asked. *(Accepted from the review edition; "24-month" dropped, since the duration is stated just below.)*

By integrating:

- the Engine (AI-powered legislative platform),
- the Bridge (targeted political coordination), and
- the Shield (health security and delivery systems),

the Hub creates a single execution flow from policy → financing → patient impact.

Within six months (Phase 1), the investment delivers: a live Hub App core, a national screening-intelligence dashboard connected to national reporting, the Legislative Ask Map, and an active caucus legislative workplan – a working solution in one country.

Within two years, the investment delivers:

[was "Within 24 months"]

- A fully operational App enabling rapid legislative action
- Data-driven political cooperation that results in concrete reforms
- Real-time dashboards and logistics systems supporting delivery within one week for routine, templated actions in pilot settings
- **Protected continuity of care across the nine partner countries, beginning with the highest-burden settings [was "for 300,000+ vulnerable patients"]**

Why now: With the regional withdrawal in 2026 from global donors, countries need practical tools to manage health systems independently. The Hub provides exactly that – speed, structure, and accountability.

This is not a traditional program, but a working model of health sovereignty – helping governments act faster, use resources more efficiently, and maintain stability in uncertain conditions. We invite partners to invest in a system that delivers measurable results within two years and a scalable model beyond.

REFERENCES

New. The note previously had no references section at all, and no inline citations. Each claim that rests on a source now carries a superscript number pointing here. All four links were checked and resolve.

1. European Centre for Disease Prevention and Control / WHO Regional Office for Europe. *Tuberculosis surveillance and monitoring in Europe 2025 – 2023 data*. Stockholm: ECDC/WHO Regional Office for Europe; 2025. [who.int](https://www.who.int)
2. Human Rights Watch. *Donor Nation Cuts to Global Health Financing Affect Millions*. 22 January 2026. [hrw.org](https://www.hrw.org)
3. Global TB Caucus. globaltbcaucus.org
4. World Health Organization. *Global strategy on digital health 2020–2027*. [who.int](https://www.who.int)